

ORLEANS PARISH CORONER'S OFFICE
DIVISION OF FORENSIC PATHOLOGY
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AUTOPSY REPORT

Case No.: PBPB-2026-034532 | Document No.: 04

Coroner Case No.: OPCO-2025-10-04-073

Decedent: DAMARI KHALIL THORNTON

DOB / Age / Sex / Race: 03/14/1991 / 34 / Male / Black

Date/Time of Death (estimated): October 4, 2025 — between 2:00 AM and 2:35 AM

Date/Time of Autopsy: October 5, 2025 — 09:14 AM

Forensic Pathologist: Vivian N. Caldwell-Beaumont, MD — Deputy Chief Medical Examiner

Pathology Assistant: Vance Doucet, F-ABMDI

Identification: Confirmed via right index fingerprint comparison (LA AFIS) and visual ID by spouse via photo (Atlanta PD)

FINAL DIAGNOSES

I. Multiple blunt force injuries to the head:

- A. Depressed comminuted fracture, right parietal bone (10.2 cm × 6.8 cm)
- B. Linear fracture extending into right temporal bone and base of skull (basilar)
- C. Subdural hematoma, right cerebral hemisphere, approximately 145 mL
- D. Cerebral contusions, right frontal and right temporal lobes
- E. Subarachnoid hemorrhage, diffuse
- F. Multiple scalp lacerations (4 total) with retained foreign material (wood splinters — see Section VIII)

II. Blunt force injuries to face:

- A. Abrasion-contusion, right zygomatic region (consistent with contact with asphalt surface)
- B. Laceration, right upper lip (1.4 cm)
- C. Fracture of right nasal bone

III. Defensive-pattern injuries:

- A. Linear contusions, dorsal aspects of both forearms (3 right, 2 left)
- B. Abrasion, dorsum of right hand, 2nd-3rd MCP joints
- C. Subungual hemorrhage, right thumb and right index finger

CAUSE OF DEATH

Cerebral hemorrhage and basilar skull fracture due to multiple blunt force injuries of the head.

MANNER OF DEATH

Homicide.

I. EXTERNAL EXAMINATION

The body is that of a normally developed, well-nourished Black male appearing the stated age of 34 years, measuring 71 inches in length and weighing 184 pounds. Rigor mortis is present in major muscle groups and is consistent with the estimated post-mortem interval. Livor mortis is fixed and posteriorly distributed (anterior face/torso pallor consistent with prone positioning). Body is received clothed; clothing has been previously inventoried by CSU (see Doc 03, items E-01, E-04, E-05).

II. HEAD AND NECK

Reflection of the scalp reveals extensive subgaleal hemorrhage overlying the right parieto-temporal region. Four discrete scalp lacerations are present, ranging from 2.1 cm to 4.8 cm. The largest (Wound A, 4.8 cm) overlies the

depressed parietal fracture and contains **three retained wood splinters** (collected for analysis — see Section VIII). The pattern, depth, and tool-mark characteristics of the wounds are consistent with multiple impacts from a heavy, elongated wooden instrument — consistent with a wooden baseball bat or similar. No firearm-related findings.

III. NEUROLOGICAL EXAMINATION

Brain weight: 1,420 g. Right cerebral hemisphere shows extensive contusion of the inferior frontal and temporal lobes. A 145 mL subdural hematoma overlies the right hemisphere. Subarachnoid hemorrhage is diffuse but most prominent over the right temporoparietal region. The brainstem shows secondary hemorrhagic disruption at the level of the pons consistent with cerebral herniation.

IV. THORACIC AND ABDOMINAL EXAMINATION

Thoracic and abdominal viscera are unremarkable. No rib fractures. No internal hemorrhage of trunk. Heart weighs 340 g; coronary arteries are patent. Lungs are well-aerated, no aspiration. Stomach contents: approximately 200 mL semi-digested material, consistent with pub-style food (fries, ground meat). Liver, spleen, kidneys, adrenals: unremarkable.

V. MUSCULOSKELETAL

Defensive injuries to the forearms as detailed under Final Diagnoses III. No fractures of the upper or lower extremities. Subungual hemorrhage of the right thumb and right index finger is consistent with active resistance, possibly grasping or clawing motion.

VI. TOXICOLOGY

Femoral blood, vitreous, and urine submitted to NMS Labs.

Ethanol (femoral blood): 0.041 g/100 mL.

Drug screen (immunoassay): negative for amphetamines, cocaine, opiates, benzodiazepines, cannabinoids, PCP.

Comprehensive panel (LC-MS/MS): negative for fentanyl analogs, synthetic cannabinoids. Caffeine present (consistent with social consumption).

VII. TIME OF DEATH ESTIMATION

Based on (a) liver temperature at scene (94.3°F at 5:21 AM with ambient 64°F), (b) early fixed lividity, (c) developing rigor at the time of scene evaluation, and (d) reported gastric contents in early-to-middle digestion, the estimated time of death is between **2:00 AM and 2:35 AM on October 4, 2025**. This range is consistent with witness timeline and security camera timestamps.

VIII. TRACE EVIDENCE / RETAINED MATERIALS

The following materials were recovered during autopsy and transferred to LA State Police Crime Lab via secure chain of custody on October 5, 2025:

- Three wood splinters from Wound A (scalp) — preliminary visual identification as oak; ash species also possible. Submitted for forensic wood identification.
- Postmortem fingernail clippings (right and left hands) — supplemental to E-06/E-07 submitted at scene.
- Hair combings (scalp, pubic) — routine.
- Postmortem blood samples (femoral, cardiac) and vitreous.

IX. OPINION

Damari Khalil Thornton, a 34-year-old Black male, died as a result of cerebral hemorrhage and basilar skull fracture caused by multiple impacts to the head with a heavy elongated wooden instrument. Defensive injuries to the upper extremities indicate that the decedent was conscious and resisting at least at the outset of the assault. The manner of death is homicide.

Forensic Pathologist Signature: _____ Vivian N. Caldwell-Beaumont, MD

Pathology Assistant: _____ V. Doucet, F-ABMDI

Chief Medical Examiner Review: Charles R. Boulanger, MD — 10/06/2025

Report Finalized: October 6, 2025